SUMMIT ORTHOPEDIC & SPINE INSTITUTE

An affiliate of Memorial Hospital of Converse County

Patient: Pulliam,Viktoriya A
DOB: 12/06/1989
Age/Sex: 35 / F
Admission Date: 06/11/25

Medical Record #: MR00140408
Account #: SG0000061755
Attending Provider: Sara Thronburg, FNP

History of Present Illness

Chief Complaint: Telephone Followup

History of Present Illness/Details: \
Miss Pulliam established care with us on October 5th, 2021. At that time, she presented in our Gillette clinic following two separate car accidents in November 2019 and February 2020. Post-accidents, she experienced pain and numbness in her left arm, with symptoms persisting since then. She reported tingling and numbness in her left arm and hand, describing her grip strength as feeling like ants biting and crawling over her hands. She had previously seen chiropractors, undergone massage therapy, and received cervical injections. A cervical MRI done at Summit Medical Center on September 30, 2021, revealed flattening and thinning of the cervical spinal cord with increased signal intensity at the C5-6 level, multilevel degenerative disk disease throughout the cervical spine, and a large diffuse disk herniation at C5-6, most prominent in the left paracentral area with cord compression. Additionally, moderate to severe neural foraminal stenosis at C6-7 was noted. She underwent a C5-6 and C6-7 anterior cervical diskectomy and fusion with Dr. Schubert at Summit Medical Center on November 4, 2021. Her postoperative symptoms subsided, leaving her with only muscle soreness and tightness. In December 2021, we discussed Kenalog injections and referred her for a second opinion regarding her thyroid. Dr. Freeman saw her for a thyroid nodule and discussed an endocrine consult and a repeat ultrasound. On February 13, 2024, she presented with returning neck pain, stiffness, muscle twitching mostly at night, bilateral arm and hand weakness, difficulty opening bottles, and bruising-type pain down her left arm. She was sent for a cervical post-myelogram CT, bilateral upper extremity EMGs, and a trial of Medrol Dosepak. Dr. Schubert reviewed her progressive neck stiffness, body jerks, bilateral upper extremity pain and paresthesias, and incidental finding of low cerebral tonsils. She was solidly fused at C5-6 and C6-7 with no significant stenosis. She was referred to Dr. Santiago, who saw her on April 9, 2024, for progressive muscle weakness and twitching, predominantly left-side weakness, cognitive concerns, and sensory level at T7 suggesting fasciculations. He recommended MRI of the brain and blood tests to determine primary demyelinating disorders such as MS. She was referred to UC Health neurology clinic and later to Oregon Clinic at OHSU to Dr. Carlson, who ordered a brachial plexus MRI. The MRI revealed canal stenosis at C5-6 with abnormal T2 signal of the left ventral cord at C5, suggestive of myelomalacia. Today, she follows up via telehealth reporting worsened symptoms including spasms and contractions in her left arm, flu-like cramping, right arm pain and numbness, shoulder blade charley horse, and difficulty sleeping due to pain.

Review of Systems

Patient reports neck pain, muscle spasms, contractions in left arm, right arm pain and numbness,

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shoulder blade charley horse, difficulty sleeping, flu-like pain/cramping, and muscle twitching. Denies sensitivity to bright lights, trouble swallowing, and snoring. Reports occasional ringing in the ears and frequent tension headaches. Coordination is reported as fine.

Allergies

Allergy/AdvReac	Type	Severity	Reaction	Status	Date / Time
Penicillins	Allergy	Unknown	unknown	Verified	04/09/24 09:22

Telehealth Visit format
Patient visit performed via: Telephone
Telehealth visit provided: in patient's home
Telehealth Verbal Consent
1. I understand that this phone conversation is considered a clinical visit with my provider over the phone.
2. I authorize and consent to this verbal examination and will answer to the best of my ability any questions my provider deems necessary.
3. I agree to be financially responsible for the charges billed by Memorial Hospital of Converse County for this examination.
Verbal consent obtained: Yes
Telehealth Time Spent
Date of Service (start): 06/11/25
Start time: 09:57
Date of Service (stop): 06/11/25
Stop time: 10:23
Total time spent: 26

Past History
Medical History (Updated 04/11/24 @ 06:22 by Angelo Santiago)

Other symptoms and signs involving cognitive functions and awareness (Chronic)
Muscle weakness (Chronic)
Fasciculations (Chronic)
Headache (Chronic)
Chronic back pain (Chronic)
Anxiety (Chronic)
HTN (hypertension)
Migraine
COVID-19 (~11/2020)
History of ectopic pregnancy

Surgical History (Updated 04/09/24 @ 09:24 by Sherrie Phillips)

Hx of hysterectomy
Hx of sinus surgery
S/P cervical spinal fusion (~11/2021)

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Family History (Updated 04/09/24 @ 09:25 by Sherrie Phillips)
No pertinent family history
Sister
High cholesterol
Father
Depression
Mother
Migraines
Mother
High blood pressure
Father

Social History (Updated 06/11/25 @ 10:28 by Sara Thronburg)
household members: spouse
housing: house
marital status: married
number of children: 2
education level: college
current occupational status: employed
current occupation: Administration
caffeine: Yes (1-2)
Smoking and tobacco status: Former smoker quit date: 01/01/07
alcohol intake frequency: holidays/special occasions only
substance use type: does not use

Exam

Imaging Review/Electrodiagnostics

Imaging Review: I re-reviewed Ms. Pullliam's cervical myelogram with post-myelogram CT performed at WY-Summit Medical Center on 3/28/2024. The craniocervical junction is normal except for some mild osteoarthritis of the C1 arch/dens articulation. The cervical spinal cord is normal in morphology except for mild narrowing of the cord at the C5-6 level. There is 4 mm of tonsillar decent with no evidence of crowding of the foramen magnum. There are metallic interbody cages in the C5-6 and C6-7 interspaces with C5-7 anterior cervical plating. There appears to be solid fusion across the C5-6 and C6-7 interspaces. There is minimal to mild degenerative changes of the C2-3 and C3-4 interspaces, mild to moderate degenerative changes in the adjacent C4-5 interspace. There is mild loss of disc space height at the adjacent C7-T1 level. There are mild diffuse disc protrusions at C3-4 and C4-5, resulting in mild central canal stenosis at the latter level. There is mild to moderate adjacent level facet arthropathy at the adjacent C7-T1 level with no more then minimal to mild facet arthropathy at the remaining cervical levels. There is mild right C3-4 neural foraminal stenosis with no significant neural foraminal stenosis at any other surgical levels. There is good posterior alignment of the cervical vertebral bodies. There is straightening of the cervical lordosis.

Ms. Pulliam underwent 4 view cervical spine x-rays at elevation medical on January 18, 2024. There are postoperative changes of the anterior plate and screw fusion hardware construct involving C5-6 and C6-7, straightening of the cervical spine is seen small endplate osteophyte complex greatest involving C4-5. Mild to space narrowing at C4-5. No dynamic instability on flexion-extension is

viewed on my read or radiology read.

Ms. Pulliam underwent a cervical MRI without contrast at elevation medical imaging. This was performed on January 18, 2024 ordered by Dr. Mansell. There is the C5-6 and C6-7 anterior cervical construct seen. There is a very mild levoscoliosis centered at the cervical thoracic junction. There is mild degenerative changes at C4-5. There is mild central canal stenosis noted at C3-4 and mild to moderate central canal stenosis at C4-5. There is also mild cord flattening due to diffuse disc protrusion resulting in mild left and moderate right neuroforaminal narrowing and facet arthropathy/ hypertrophy worse on the right than the left. There is also mild cord flattening at C6-7 due to a protruding disc eccentric towards the left. Neuroforaminal stenosis is noted. There is cerebral tonsillitis projecting 6.5 mm inferior to the foramen magnum. This would need a separate brain MRI considering this is an incidental finding to exclude intracranial hypotension as cause for the low position of the cerebral tonsils. No syrinx of the cervical cord is seen. I have discussed all findings with patient.

Electrodiagnostics: I reviewed Ms. Pulliam's EMG/NCV studies of the bilateral upper extremities performed by Dr. Santiago on 3/28/2024. The study was interpreted as demonstrating electrodiagnostic evidence of a left C6 radiculopathy, with no evidence of brachial plexopathy or entrapment neuropathy.

Impression

Since her last visit, Miss Pulliam reports progressive worsening of symptoms including spasms and contractions in her left arm, right arm pain and numbness, shoulder blade charley horse, and difficulty sleeping due to pain. She also reports flu-like cramping and muscle twitching. The right arm pain and numbness have become more severe than her initial left arm symptoms. She experiences constant tightness and squeezing pain in her neck, significantly limiting her range of motion. She has tried meloxicam and steroids, which have provided minimal relief. Occasionally, she resorts to taking leftover hydrocodone from her hysterectomy due to severe pain. She reports muscle weakness, particularly in her arms, and occasional flu-like cramps in her legs. Coordination remains unaffected, but she frequently experiences tension headaches and occasional ringing in the ears.

Physical Therapy

The patient has a history of engaging in physical therapy modalities such as chiropractic care, massage therapy, and cervical injections following her car accidents in November 2019 and February 2020. However, there is no recent mention of physical therapy being part of her current treatment plan. She expressed frustration and fatigue with ongoing medical interventions, including physical therapy, indicating a need to evaluate the effectiveness of past treatments and consider alternative approaches.

Medical Decision Making

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Viktoriya Pulliam is a 35-year-old female with a complex medical history involving multiple car accidents and subsequent cervical spine issues, including multilevel degenerative disk disease and a large diffuse disk herniation at C5-6 with cord compression. She underwent a C5-6 and C6-7 anterior cervical diskectomy and fusion in November 2021, which initially resolved her symptoms but has since developed new and worsening symptoms. These include neck stiffness, muscle twitching,

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weakness in bilateral arms and hands, dropping objects, difficulty opening bottles, and bruising-type pain down her left arm. Recent imaging studies suggest canal stenosis and abnormal T2 signal in the cervical cord, consistent with previous findings of myelomalacia. Her symptoms have progressed to involve her right arm, with severe pain, tingling, numbness, and muscle spasms, significantly impacting her quality of life and ability to sleep. Given her complex presentation, including possible Chiari malformation and fasciculations, a thorough evaluation by neurology and neurosurgery at an academic center is warranted to rule out neuromuscular disorders and further assess her cervical spine condition. Immediate management will include a cervical MRI without contrast, muscle relaxers, and pain medication to alleviate her symptoms.

Procedure Documentation

During today's telehealth visit, I reviewed Viktoriya Pulliam's extensive medical history and discussed her recent brachial plexus MRI findings, which showed canal stenosis at C5-6 with abnormal T2 signal of the left ventral cord at C5. Additionally, the MRI of the left brachial plexus was normal. Viktoriya reported worsening symptoms including muscle spasms, numbness, and pain in her right arm and shoulder blade, which have been affecting her sleep and daily activities. We discussed the importance of obtaining a new cervical MRI without contrast to compare with her previous imaging from January 18, 2024, to evaluate any changes or new herniations. We also discussed referring her to UC Health for a neurology consultation to rule out neuromuscular disorders and to the neurosurgery department to evaluate her Chiari malformation. Medications prescribed today include methocarbamol for muscle spasms and a short course of hydrocodone for pain management.

Treatment Risks & Benefits

The benefits of obtaining a new cervical MRI include the ability to accurately assess any changes or new developments in Viktoriya's cervical spine that may be contributing to her symptoms. This imaging will help guide further treatment decisions, including potential interventions such as injections or surgery if necessary. The risks associated with the MRI are minimal, primarily involving discomfort during the procedure and the potential for allergic reactions to the contrast, which is why we opted for a non-contrast MRI. The benefits of consulting with UC Health neurology and neurosurgery departments include receiving specialized evaluations for neuromuscular disorders and Chiari malformation, which may provide further insights into her symptoms and guide appropriate treatment plans. The risks of these consultations are minimal but may include the inconvenience of travel and potential delays in diagnosis. The prescribed medications, methocarbamol and hydrocodone, offer benefits in managing muscle spasms and pain, respectively. Risks include potential side effects such as drowsiness, dizziness, and the risk of dependency with hydrocodone. Viktoriya was advised not to drive while taking methocarbamol due to its sedative effects.

Assessment & Plan

Problems

1. History of cervical spinal arthrodesis Z98.1
2. Radicular pain of left upper extremity M79.2
3. Bilateral arm weakness R29.898
4. Muscle twitching R25.3
5. Weakness of both legs R29.898
6. Paresthesia and pain of both upper extremities R20.2; M79.601; M79.602

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7. Tingling of both upper extremities R20.2
8. Degeneration of intervertebral disc of cervical region with osteophyte of cervical vertebra M50.30; M25.78
9. Facet arthritis of cervical region M47.812
10. Cervicogenic headache G44.86
11. Fasciculations R25.3
12. Abnormal coordination R27.8

Plan

The immediate plan is to order a cervical MRI without contrast and ensure the radiologist compares it to the January 18, 2024, MRI to evaluate any changes or new herniations.

Viktoriya will be referred to both UC Health neurology and neurosurgery departments for further evaluation of neuromuscular disorders and her Chiari malformation.

We will continue with the prescribed methocarbamol for muscle spasms and a short course of hydrocodone for pain management. Mary will coordinate the scheduling of the MRI and follow-up appointments. Depending on the MRI results, we will consider additional interventions such as injections or surgical consultations.

Viktoriya will follow up with me or Dr. Schubert once the MRI is completed. We will reassess her symptoms and adjust the treatment plan accordingly. We discussed the possibility of exploring physical therapy options based on the MRI findings.

Viktoriya expressed concerns about the financial burden of repeated imaging and consultations, and we will strive to keep her care as local and convenient as possible. Her pain levels and symptom progression will be closely monitored, and we will continue to support her through this challenging period.

Documentation today was completed using an AI software system, and/or voice to text software recognition system. Grammatical errors may have occurred and should be taken into consideration accordingly.

Medications

New:

methocarbamol Take 1 tablet every 8 hours as needed for muscle spasms	750 mg PO TID 90 tabs 0RF
hydrocodone-acetaminophen 7.5-325 mg Take 1 tablet every four to six hours as needed for pain	1 tab PO Q4-6H PRN pain 20 tabs 0RF

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<Electronically signed by Sara Thronburg, FNP>
06/11/25 1028

<Signature on File>

cc:

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Patient: Pulliam,Viktoriya A
DOB: 12/06/1989
Age/Sex: 35 / F
Admission Date: 07/18/25

Medical Record #: MR00140408
Account #: SG0000066921
Attending Provider: Sara Thronburg, FNP

History of Present Illness

Chief Complaint: Telephone Followup

History of Present Illness/Details: Miss Pulliam established care with us on October 5th, 2021. At that time, she presented in our Gillette clinic following two separate car accidents in November 2019 and February 2020. Post-accidents, she experienced pain and numbness in her left arm, with symptoms persisting since then. She reported tingling and numbness in her left arm and hand, describing her grip strength as feeling like ants biting and crawling over her hands. She had previously seen chiropractors, undergone massage therapy, and received cervical injections. A cervical MRI done at Summit Medical Center on September 30, 2021, revealed flattening and thinning of the cervical spinal cord with increased signal intensity at the C5-6 level, multilevel degenerative disk disease throughout the cervical spine, and a large diffuse disk herniation at C5-6, most prominent in the left paracentral area with cord compression. Additionally, moderate to severe neural foraminal stenosis at C6-7 was noted. She underwent a C5-6 and C6-7 anterior cervical diskectomy and fusion with Dr. Schubert at Summit Medical Center on November 4, 2021. Her postoperative symptoms subsided, leaving her with only muscle soreness and tightness. In December 2021, we discussed Kenalog injections and referred her for a second opinion regarding her thyroid. Dr. Freeman saw her for a thyroid nodule and discussed an endocrine consult and a repeat ultrasound. On February 13, 2024, she presented with returning neck pain, stiffness, muscle twitching mostly at night, bilateral arm and hand weakness, difficulty opening bottles, and bruising-type pain down her left arm. She was sent for a cervical post-myelogram CT, bilateral upper extremity EMGs, and a trial of Medrol Dosepak. Dr. Schubert reviewed her progressive neck stiffness, body jerks, bilateral upper extremity pain and paresthesias, and incidental finding of low cerebral tonsils. She was solidly fused at C5-6 and C6-7 with no significant stenosis. She was referred to Dr. Santiago, who saw her on April 9, 2024, for progressive muscle weakness and twitching, predominantly left-side weakness, cognitive concerns, and sensory level at T7 suggesting fasciculations. He recommended MRI of the brain and blood tests to determine primary demyelinating disorders such as MS. She was referred to UC Health neurology clinic and later to Oregon Clinic at OHSU to Dr. Carlson, who ordered a brachial plexus MRI. The MRI revealed canal stenosis at C5-6 with abnormal T2 signal of the left ventral cord at C5, suggestive of myelomalacia.

The patient, Viktoriya Pulliam, has been experiencing worsening symptoms related to her Chiari malformation and myelomalacia. She reports that she initially felt significant relief following her spinal surgery but began experiencing increased pain approximately one year postoperatively. Recently, her symptoms have dramatically worsened, especially after an episode of flu. She describes severe pain, tingling, and numbness in her right arm, muscle spasms, and difficulty sleeping. Despite

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taking high doses of Advil and Tylenol, as well as Lyrica 50 mg at bedtime, her pain remains poorly controlled. She is concerned about the potential for GI bleeding due to her high intake of NSAIDs and acetaminophen. The patient has been referred to neurology for further evaluation and is considering pain management options such as a pain pump or stimulator.

She reports severe pain, tingling, and numbness in her right arm, muscle spasms, and difficulty sleeping. She experiences occasional numbness and tingling in her legs. The pain medication helps to some extent but does not provide complete relief. She also reports feeling nervous about a potential GI bleed due to high doses of Advil and Tylenol. The patient experiences stiffness and pain that becomes more manageable with medication but never fully resolves. She is concerned about the impact of these symptoms on her daily activities, especially while caring for her two-year-old daughter.

Medical Record Review: 6/19/25-
"Today, I conducted a telephone follow-up with Viktoriya Pulliam. During our call, we discussed her recent cervical MRI results and her ongoing symptoms following her C5-6 and C6-7 anterior cervical diskectomy and fusion on November 21st, 2021. Despite the initial resolution of her symptoms post-surgery, she has developed new and worsening symptoms, including neck stiffness, muscle twitching, weakness in her bilateral arms and hands, difficulty opening bottles, bruising-type pain down her left arm, severe pain, tingling, numbness, muscle spasms, and significant impact on her quality of life and ability to sleep. Recent imaging studies suggested canal stenosis and abnormal T2 signal in the cervical cord consistent with previous findings of myelomalacia. The radiologist compared the latest MRI to the brachial plexus MRI instead of the cervical MRI. Upon review with Dr. Schubert, we determined that the artifact present in the MRI was too significant to accurately assess cord deformation. Therefore, I recommended a cervical post-myelogram CT to better evaluate the amount of stenosis and any nerve compression.

Treatment Risks & Benefits

I discussed the option of undergoing a cervical post-myelogram CT with Viktoriya to assess the amount of stenosis and nerve compression. This procedure involves the injection of contrast dye into the cerebrospinal fluid (CSF) followed by a CT scan to visualize the spinal canal and neural foramen without the artifact present in the MRI. The benefits of this procedure include a more accurate assessment of the stenosis and the potential identification of nerve compression, which could explain her worsening symptoms. The risks associated with the procedure include potential discomfort from the injection, possible allergic reactions to the contrast dye, and the inherent risks of radiation exposure from the CT scan. Additionally, I discussed the option of starting her on Lyrica (pregabalin) at a dosage of 50 mg to manage her neuropathic pain, given her history of cord myelomalacia. The benefits of Lyrica include its efficacy in treating nerve pain and fibromyalgia, with quicker absorption and fewer interactions compared to gabapentin. The risks include potential side effects such as dizziness, drowsiness, and weight gain. I advised her to monitor for any adverse effects and report them promptly.

Plan

The immediate plan for Viktoriya includes scheduling a cervical post-myelogram CT to better assess

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the amount of stenosis and any nerve compression.

I will have the office staff, Mary and Michelle, follow up with her to ensure the referral to UC Health Neurology and Neurosurgery Departments is completed and to schedule the myelogram CT.

Additionally, I will prescribe Lyrica (pregabalin) at 50 mg to be taken at night to help manage her neuropathic pain. I advised Viktoriya to continue taking methocarbamol for muscle spasms and hydrocodone for pain management as needed.

She should monitor her symptoms and report any changes or adverse effects from the new medication. We will follow up after the myelogram CT results are available to discuss further treatment options and any necessary adjustments to her medication regimen.

I also recommended she avoid chiropractic interventions and consider physical therapy for symptomatic relief.-Please send order for PT"

Allergies

Allergy/AdvReac	Type	Severity	Reaction	Status	Date / Time
Penicillins	Allergy	Unknown	unknown	Verified	04/09/24 09:22

Telehealth Visit format
Patient visit performed via: Telephone
Telehealth visit provided: in patient's home
Telehealth Verbal Consent
1. I understand that this phone conversation is considered a clinical visit with my provider over the phone.
2. I authorize and consent to this verbal examination and will answer to the best of my ability any questions my provider deems necessary.
3. I agree to be financially responsible for the charges billed by Memorial Hospital of Converse County for this examination.
Verbal consent obtained: Yes
Telehealth Time Spent
Date of Service (start): 07/18/25
Start time: 09:32
Date of Service (stop): 07/18/25
Stop time: 09:54
Total time spent: 22

Past History
Medical History (Updated 04/11/24 @ 06:22 by Angelo Santiago)

Other symptoms and signs involving cognitive functions and awareness (Chronic)
Muscle weakness (Chronic)
Fasciculations (Chronic)
Headache (Chronic)
Chronic back pain (Chronic)

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Anxiety (Chronic)
HTN (hypertension)
Migraine
COVID-19 (~11/2020)
History of ectopic pregnancy

Surgical History (Updated 04/09/24 @ 09:24 by Sherrie Phillips)

Hx of hysterectomy
Hx of sinus surgery
S/P cervical spinal fusion (~11/2021)

Family History (Updated 04/09/24 @ 09:25 by Sherrie Phillips)
No pertinent family history
Sister
High cholesterol
Father
Depression
Mother
Migraines
Mother
High blood pressure
Father

Social History (Updated 07/18/25 @ 10:28 by Sara Thronburg)
household members: spouse
housing: house
marital status: married
number of children: 2
education level: college
current occupational status: employed
current occupation: Administration
caffeine: Yes (1-2)
Smoking and tobacco status: Former smoker quit date: 01/01/07
alcohol intake frequency: holidays/special occasions only
substance use type: does not use

Exam

Imaging Review/Electrodiagnostics
Imaging Review:
Cervical MRI without contrast performed on June 17, 2025
- Comparison was incorrectly made to the brachial plexus MRI instead of the cervical MRI
  - C2-3: No foraminal stenosis
  - C3-4: No canal stenosis; mild to moderate neural foraminal narrowing on the right and mild on the left due to facet arthropathy
  - C4-5: Disk osteophyte complex causing mild canal stenosis and moderate right and mild left neural foraminal stenosis

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- C5-6: Significant artifact present; moderate canal stenosis noted but requires cervical spine myelogram CT for proper assessment; deforming of the ventral cord noted but deemed unreliable due to artifact
- C5-6 and C6-7: No neural foraminal stenosis or central canal stenosis
- Radiologist's impression: Small foci high T2 signal within the left ventral cord at C5 level and within the left and right central cord at the C5-6 level, likely myelomalacia.

I re-reviewed Ms. Pullliam's cervical myelogram with post-myelogram CT performed at WY-Summit Medical Center on 3/28/2024. The craniocervical junction is normal except for some mild osteoarthritis of the C1 arch/dens articulation. The cervical spinal cord is normal in morphology except for mild narrowing of the cord at the C5-6 level. There is 4 mm of tonsillar decent with no evidence of crowding of the foramen magnum. There are metallic interbody cages in the C5-6 and C6-7 interspaces with C5-7 anterior cervical plating. There appears to be solid fusion across the C5-6 and C6-7 interspaces. There is minimal to mild degenerative changes of the C2-3 and C3-4 interspaces, mild to moderate degenerative changes in the adjacent C4-5 interspace. There is mild loss of disc space height at the adjacent C7-T1 level. There are mild diffuse disc protrusions at C3-4 and C4-5, resulting in mild central canal stenosis at the latter level. There is mild to moderate adjacent level facet arthropathy at the adjacent C7-T1 level with no more then minimal to mild facet arthropathy at the remaining cervical levels. There is mild right C3-4 neural foraminal stenosis with no significant neural foraminal stenosis at any other surgical levels. There is good posterior alignment of the cervical vertebral bodies. There is straightening of the cervical lordosis.

Ms. Pulliam underwent 4 view cervical spine x-rays at elevation medical on January 18, 2024. There are postoperative changes of the anterior plate and screw fusion hardware construct involving C5-6 and C6-7, straightening of the cervical spine is seen small endplate osteophyte complex greatest involving C4-5. Mild to space narrowing at C4-5. No dynamic instability on flexion-extension is viewed on my read or radiology read.

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Impression

Interval History

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Since her last visit, the patient reports worsening symptoms, including increased pain, tingling, and numbness in her right arm, and muscle spasms in her left arm. She describes difficulty sleeping and managing her pain despite taking high doses of Advil, Tylenol, and Lyrica. She has experienced severe discomfort after a recent episode of flu, which seems to have exacerbated her symptoms. The patient has been referred to neurology for further evaluation and is considering advanced pain management options such as a pain pump or stimulator. She is scheduled for a cervical post-myelogram CT at the end of July and has an upcoming appointment with a pain management specialist in Rapid City.

Physical Therapy

Physical therapy was not specifically mentioned during the telehealth consultation. The patient did not discuss any current physical therapy regimen or past experiences with physical therapy. No recommendations or referrals for physical therapy were made during this session.

Imaging

- MRI reviewed on June 17th, 2025, showing myelomalacia that was present before the original surgery
- Cervical post-myelogram CT scheduled for the end of July (28th or 29th)
- Myelogram to be performed to rule out any missed issues or artifacts not visible on MRI

Medical Decision Making

The patient, Viktoriya Pulliam, a 35-year-old female, continues to experience severe pain, tingling, and numbness primarily in her right arm, along with muscle spasms in her left arm and occasional numbness and tingling in her legs. These symptoms have worsened significantly over the past two months. The patient reports that while pain medications, including Lyrica, Advil, and Tylenol, provide some relief, they do not completely alleviate her symptoms, and she is concerned about the long-term effects of these medications on her liver and kidneys. The patient has also been experiencing difficulty sleeping due to the pain and discomfort. Despite a successful fusion surgery, the patient is now dealing with new and worsening symptoms, possibly exacerbated by recent illness. A minimal Chiari malformation was identified but deemed not to be the primary issue by CU Health. The patient has also been referred to UC Health neurology for further evaluation and is awaiting scheduling to rule out a neuromuscular or motor neuron disorder. She is also scheduled to see a pain management specialist to discuss options such as a pain pump or stimulator. An increase in Lyrica dosage to 50 mg twice daily has been prescribed to manage symptoms more effectively. The patient has not found muscle relaxers beneficial due to side effects and prefers to avoid them. Further imaging with a myelogram and post-myelogram CT is planned to ensure no critical findings are missed.

Several treatment options were discussed with the patient. Increasing the dosage of Lyrica was considered, starting with 50 mg twice daily to manage pain and tingling symptoms. The benefits of this approach include potentially better symptom control and improved sleep. The risks involve possible side effects such as drowsiness, which the patient initially experienced but has since diminished. Another option discussed was the use of muscle relaxers, although the patient expressed concerns about side effects such as sinus congestion and headaches. The patient also mentioned her discomfort with oral pain medications like hydrocodone due to their impact on her ability to function daily, particularly while caring for her two-year-old daughter. The potential for gastrointestinal issues

from high doses of Advil and Tylenol was noted, as the patient has been experiencing stomach discomfort and concerns about a possible GI bleed. Additionally, the patient has an upcoming appointment with a pain management specialist to explore options such as a pain pump or stimulator. The benefits of these interventions include bypassing the liver and brain to provide direct pain relief, potentially reducing the need for oral medications. However, the risks include surgical complications and the possibility of the stimulator causing a vibration sensation without addressing numbness. The patient expressed openness to these options despite her initial apprehension, recognizing the need for effective long-term pain management.

Assessment & Plan

Problems

1. History of cervical spinal arthrodesis Z98.1
2. Nerve pain M79.2
3. Bilateral arm pain M79.601; M79.602
4. Paresthesia and pain of both upper extremities R20.2; M79.601; M79.602
5. Abnormal coordination R27.8
6. Tingling of both upper extremities R20.2
7. Fasciculations R25.3
8. Weakness of both legs R29.898
9. Bilateral arm weakness R29.898
10. Muscle twitching R25.3

Plan

The immediate plan is to increase the patient's Lyrica dosage to 50 mg twice daily to better manage her pain and tingling symptoms. She will continue to avoid muscle relaxers due to previous adverse effects, and instead focus on optimizing her Lyrica regimen. The patient will follow up with UC Health Neurology to rule out any neuromuscular disorders, as her symptoms have worsened despite initial post-surgical relief and stable fusion findings.

Additionally, she will undergo a cervical post-myelogram CT at the end of July to ensure there are no artifacts affecting the current imaging results. The patient has an appointment with a pain management specialist in Rapid City on Tuesday to discuss potential interventions such as a pain pump or stimulator. It is important to monitor her kidney function and gastrointestinal health due to the high doses of Advil and Tylenol she has been taking. The patient will call UC Health Neurology to expedite the scheduling of her appointment and will follow up with our clinic after her myelogram for further evaluation and management. The goal is to provide effective pain relief while minimizing side effects and ensuring her ability to care for her young daughter.

Documentation today was completed using an AI software system, and/or voice to text software recognition system. Grammatical errors may have occurred and should be taken into consideration accordingly.

Patient: Pulliam,Viktoriya A
DOB: 12/06/1989
Age/Sex: 35 / F
Admission Date: 07/18/25

Medical Record #: MR00140408
Account #: SG0000066921
Attending Provider: Sara Thronburg, FNP

Medications

New:

pregabalin (Lyrica) 50 mg PO BID 60 caps 0RF MDD 2
Take 1 cap twice daily

Discontinued:

pregabalin (Lyrica) 50 mg PO BEDTIME 30 caps 0RF MDD 1
Take 1 cap at bedtime
Discontinued Reason: Complete

methocarbamol 750 mg PO TID 90 tabs 0RF
Take 1 tablet every 8 hours as needed for muscle spasms
Discontinued Reason: Complete

meloxicam 15 mg PO DAILY 30 tabs 0RF MDD 1
Take 1 tablet daily; do not take with other NSAIDS
Discontinued Reason: Complete

<Electronically signed by Sara Thronburg, FNP>
07/18/25 1028

<Signature on File>

cc:

SUMMIT ORTHOPEDIC & SPINE INSTITUTE

An affiliate of Memorial Hospital of Converse County

Patient: Pulliam,Viktoriya A
DOB: 12/06/1989
Age/Sex: 35 / F
Admission Date: 07/31/25

Medical Record #: MR00140408
Account #: SG0000065786
Attending Provider: Sara Thronburg, FNP

History of Present Illness

Chief Complaint: Telephone Followup

History of Present Illness/Details:
Viktoriya Pulliam is a very pleasant 35-year-old female who is following up after a myelogram performed at Campbell County on July 29th, 2025. She has a history of anterior cervical diskectomy and fusion at C5-6 and C6-7. The myelogram revealed mature bony bridging throughout the fusion construct with no evidence of hardware loosening or complication. There is straightening of the cervical lordosis, possibly due to muscle spasm, and mild to moderate foraminal stenosis at C4-5 due to a disc osteophyte complex causing mild flattening of the cord. At C5-6, there is calcification of the posterior longitudinal ligament causing deformity of the ventral cord and moderate central canal stenosis. There is residual myelomalacia from a previous accident and herniation. She reports seeing a pain management specialist on August 18th, who prescribed baclofen, which has been effective in managing her symptoms. However, she has experienced unusual side effects, including congestion, a burning sensation in her tongue, and plugged ears. She has never had allergies before and is unsure if these symptoms are related to the medication.

Review of Systems

Patient reports congestion, a burning sensation in her tongue, and plugged ears since starting baclofen. She denies having a history of allergies. She experiences drowsiness, confusion, dizziness, and weakness, which are common side effects of baclofen. She also has persistent left arm spasms and stiffness, which were noted by her pain management specialist during an examination. She has no history of similar symptoms in her right arm.

Allergies

Allergy/AdvReac	Type	Severity	Reaction	Status	Date / Time
Penicillins	Allergy	Unknown	unknown	Verified	04/09/24 09:22

Telehealth Visit format
Patient visit performed via: Telephone
Telehealth visit provided: in patient's home
Telehealth Verbal Consent

1541 Diamond Drive, Casper WY 82601 www.ConverseHospital.com 535 E Main, Ste A Lander WY 82520

Patient: Pulliam,Viktoriya A
DOB: 12/06/1989
Age/Sex: 35 / F
Admission Date: 07/31/25

Medical Record #: MR00140408
Account #: SG0000065786
Attending Provider: Sara Thronburg, FNP

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1. I understand that this phone conversation is considered a clinical visit with my provider over the phone.
2. I authorize and consent to this verbal examination and will answer to the best of my ability any questions my provider deems necessary.
3. I agree to be financially responsible for the charges billed by Memorial Hospital of Converse County for this examination.

Verbal consent obtained: Yes
Telehealth Time Spent
Date of Service (start): 07/31/25
Start time: 09:05
Date of Service (stop): 07/31/25
Stop time: 09:20
Total time spent: 15

Past History
Medical History (Updated 04/11/24 @ 06:22 by Angelo Santiago)

Other symptoms and signs involving cognitive functions and awareness (Chronic)
Muscle weakness (Chronic)
Fasciculations (Chronic)
Headache (Chronic)
Chronic back pain (Chronic)
Anxiety (Chronic)
HTN (hypertension)
Migraine
COVID-19 (~11/2020)
History of ectopic pregnancy

Surgical History (Updated 04/09/24 @ 09:24 by Sherrie Phillips)

Hx of hysterectomy
Hx of sinus surgery
S/P cervical spinal fusion (~11/2021)

Family History (Updated 04/09/24 @ 09:25 by Sherrie Phillips)
No pertinent family history
Sister
High cholesterol
Father
Depression
Mother
Migraines
Mother
High blood pressure
Father

Social History (Updated 07/31/25 @ 09:30 by Sara Thronburg)
household members: spouse

Patient: Pulliam,Viktoriya A
DOB: 12/06/1989
Age/Sex: 35 / F
Admission Date: 07/31/25

Medical Record #: MR00140408
Account #: SG0000065786
Attending Provider: Sara Thronburg, FNP

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housing: house
marital status: married
number of children: 2
education level: college
current occupational status: employed
current occupation: Administration
caffeine: Yes (1-2)
Smoking and tobacco status: Former smoker quit date: 01/01/07
alcohol intake frequency: holidays/special occasions only
substance use type: does not use

Exam

Imaging Review/Electrodiagnostics

Imaging Review: Ms. Pulliam underwent a cervical post myelogram CT on 7/19/2025
- Anterior cervical diskectomy and fusion hardware construct at C5-6 and C6-7 with good mature bony bridging and no evidence of hardware loosening or complication
- Straightening of the cervical lordosis, possibly due to muscle spasm
- No spondylolisthesis observed
- Intrathecal contrast showed no masses, and fluid was observed around the cord
- C2-3 level appeared normal with no issues
- C3-4 level showed no canal stenosis, mild to moderate right foraminal stenosis, and facet arthritis, right greater than left
- C4-5 level showed a disc protrusion with a disc osteophyte complex causing mild to moderate stenosis and mild flattening of the thecal sac.
- C5-6 level showed calcification of the posterior longitudinal ligament, causing deformity of the thecal sac and no more than moderate central canal stenosis
- C6-7 level appeared normal with no issues
- C7-T1 level showed mild facet arthritis with no canal or foraminal stenosis

Impression

Since her last visit, Viktoriya has seen a pain management specialist who prescribed baclofen, which has been effective in managing her symptoms. However, she has experienced unusual side effects, including congestion, a burning sensation in her tongue, and plugged ears. She has never had allergies before and is unsure if these symptoms are related to the medication. She continues to experience persistent left arm spasms and stiffness. Her neurology appointment has not been confirmed yet, and she is considering contacting the University of Utah for further evaluation.

Diagnostic Testing
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1. Myelogram performed on July 29th, 2025 at Campbell County, showing no severe stenosis and fluid around the cord
2. Pain management specialist consultation on August 18th, 2025, resulting in the prescription of baclofen
3. Dr. Santiago's evaluation in April 2024, including MRI of the brain to rule out primary demyelinating disorder and various blood tests (CBC, CMP, ESR, ANA, TSH, GAD, CPEK, aldolase)
4. UC Health neurology appointment scheduled for December 2, 2024, which may have been

rescheduled or missed

Medical Decision Making

The patient, Viktoriya Pulliam, a 35-year-old female, is following up after a myelogram performed on July 29th, 2025. Imaging studies show stable anterior cervical diskectomy and fusion at C5-6 and C6-7 with no hardware complications. Mild to moderate foraminal stenosis and facet arthritis are noted at various levels, with no severe canal stenosis or compressive pathology observed. The patient has a history of myelomalacia secondary to a prior accident, contributing to her current symptoms. She is experiencing significant muscle spasm and stiffness, particularly in her left arm, which has been partially alleviated by baclofen. However, she reports unusual side effects, including congestion and a burning sensation in her mouth and ears, potentially linked to the medication. Neurological symptoms, including progressive muscle weakness and sensory issues, are being further evaluated with referrals to neurology and consideration of a pain pump trial. Coordination with her pain management specialist and further follow-up with neurology are recommended to rule out other neuromuscular disorders and optimize her treatment plan.

Procedure Documentation

The patient, Viktoriya Pulliam, underwent a myelogram at Campbell County on July 29th, 2025. The myelogram revealed several findings including an anterior cervical diskectomy and fusion hardware construct at C5-6 and C6-7 with good mature bony bridging throughout the fusion construct and no evidence of hardware loosening or complications. The imaging showed straightening of the cervical lordosis, potentially due to muscle spasm, and no spondylolisthesis. The intrathecal contrast demonstrated good distribution of cerebrospinal fluid (CSF) without any masses. At C2-3, the level appears normal with no issues. At C3-4, there is no canal stenosis but mild to moderate right foraminal stenosis due to facet arthritis. At C4-5, there is a disc protrusion with a disc osteophyte complex causing mild to moderate stenosis and mild flattening of the thecal sac and this was confirmed with Dr. Schubedrt. There is mild facet arthritis throughout the left side and mild right neural foraminal narrowing. At C5-6, there is calcification of the posterior longitudinal ligament causing deformity of the ventral cord and moderate central canal stenosis, though the CSF flow is preserved. At C6-7, the level appears normal. At C7-T1, there is mild facet arthritis with no canal or foraminal stenosis. Overall, the findings indicate mild to moderate degenerative changes with no severe stenosis.

Treatment Risks & Benefits

The patient has been prescribed baclofen by her pain management specialist on July 18th, 2025, to address muscle spasms, particularly in her left arm. The patient reports significant relief from spasms but has experienced unusual side effects including congestion, a burning sensation in her tongue, and plugged ears. Common side effects of baclofen include drowsiness, confusion, dizziness, and weakness. The patient is hopeful that these systemic symptoms will be alleviated once a pain pump is implemented, as suggested by her pain management specialist. The patient is scheduled for a pain pump trial on August 18th, 2025. The potential benefits of the pain pump include targeted relief of muscle spasms and reduction of systemic side effects. The risks associated with the pain pump include infection, catheter-related complications, and potential hardware issues. The patient has also

Patient: Pulliam,Viktoriya A
DOB: 12/06/1989
Age/Sex: 35 / F
Admission Date: 07/31/25

Medical Record #: MR00140408
Account #: SG0000065786
Attending Provider: Sara Thronburg, FNP

been advised to follow up with neurology to rule out neuromuscular disorders and ensure comprehensive care.

Assessment & Plan

Problems

1. History of cervical spinal arthrodesis Z98.1
2. Fasciculations R25.3
3. Nerve pain M79.2
4. Bilateral arm pain M79.601; M79.602
5. Muscle twitching R25.3
6. Bilateral arm weakness R29.898
7. Weakness of both legs R29.898
8. Paresthesia and pain of both upper extremities R20.2; M79.601; M79.602
9. Abnormal coordination R27.8
10. Tingling of both upper extremities R20.2

Plan

The plan for Viktoriya Pulliam includes continuing the current prescription of baclofen to manage muscle spasms while monitoring for side effects. The patient will proceed with the pain pump trial scheduled for August 18th, 2025, to evaluate the efficacy of targeted medication delivery. She is advised to follow up with neurology to rule out any neuromuscular disorders, given her history of progressive muscle weakness, cognitive concerns, and sensory issues. The patient should call UC Health to confirm her neurology appointment and consider seeking a second opinion at an academic center such as the University of Utah if necessary. Additionally, she is encouraged to use the patient portal to access her medical records and ensure her pain management specialist has all necessary information. The patient should also try over-the-counter allergy medications like Zyrtec D to address her congestion symptoms and report any changes. Follow-up appointments will be scheduled as needed to monitor her condition and adjust the treatment plan accordingly.

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<Electronically signed by Sara Thronburg, FNP>
07/31/25 0930

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Patient: Pulliam,Viktoriya A
DOB: 12/06/1989
Age/Sex: 35 / F
Admission Date: 07/31/25

Medical Record #: MR00140408
Account #: SG0000065786
Attending Provider: Sara Thronburg, FNP

cc:

Pulliam,Viktoriya A
35 F 12/06/1989
DEP AMB

07/31/25 09:00 - 07/31/25 09:42
Spec Indicators: None
AETNA4 AMB

SG0000065786
208.00 AR

MR00140408
Z00102865

Inquiry by Transaction

+ −

Svc Date	Chg Code	Txn #	Quantity	Txn Amt
07/31/25 09:00	99213	2	1	208.00

Batch: 09/11/25 PENDCHGS #63
Description: ESTAB PT LVL 3 =>20MIN
User: SWITZER,TAMMY
Prof Comp: 208.00
Order Type: Standard
Master Charge Code: 510000213

Claim Type	Rate Schedule	Alt Codes	CDM Modifiers	CDM Multipliers
Standard		CPT-4 99213		

Txn Modifiers
Override CDM: Yes
Mod 1: 93
Mod 2:
Mod 3:
Mod 4:
Mod 5:

Diagnosis Codes
Dx 1: Z98.1
Dx 2: R25.3
Dx 3: M79.2
Dx 4: M79.601

Rx Data
Prescription Number:
National Drug Code:
Unit of Measure:
Quantity:

Providers
Performing Provider: SARG833
NPI Number: 1225662216
License Number: 45497
Ordering/Referring Provider:
NPI Number:
License Number:

Txn Insurance Order

Ins	Prorated
1 AETNA4	208.00
2 SP	0.00

Pulliam,Viktoriya A
35 F 12/06/1989
DEP AMB

07/18/25 09:43 - 07/18/25 10:24
Spec Indicators: None
AETNA4 AMB

SG0000066921
208.00 AR

MR00140408
Z00102865

Inquiry by Transaction

+ −

Svc Date	Chg Code	Txn #	Quantity	Txn Amt
07/18/25 09:43	99213	2	1	208.00

Batch: 09/10/25 PENDCHGS #109
Description: ESTAB PT LVL 3 =>20MIN
User: SWITZER,TAMMY
Prof Comp: 208.00
Order Type: Standard
Master Charge Code: 510000213

Claim Type	Rate Schedule	Alt Codes	CDM Modifiers	CDM Multipliers
Standard		CPT-4 99213		

Txn Modifiers
Override CDM: Yes
Mod 1: 93
Mod 2:
Mod 3:
Mod 4:
Mod 5:

Diagnosis Codes
Dx 1: M79.2
Dx 2: M79.601
Dx 3: M79.602
Dx 4: R20.2

Rx Data
Prescription Number:
National Drug Code:
Unit of Measure:
Quantity:

Providers
Performing Provider: SARG833
NPI Number: 1225662216
License Number: 45497
Ordering/Referring Provider:
NPI Number:
License Number:

Txn Insurance Order

Ins	Prorated
1 AETNA4	208.00
2 SP	0.00

Pulliam,Viktoriya A
35 F 12/06/1989
DEP AMB

06/11/25 09:58 - 06/11/25 12:30
Spec Indicators: None
AETNA4 AMB

SG0000061755
194.00 AR

MR00140408
Z00102865

Inquiry by Transaction

+ −

Svc Date	Chg Code	Txn #	Quantity	Txn Amt
06/11/25 09:58	99213	2	1	194.00

Batch: 08/13/25 PENDCHGS #101
Description: ESTAB PT LVL 3 =>20MIN
User: SWITZER,TAMMY
Prof Comp: 194.00
Order Type: Standard
Master Charge Code: 510000213

Claim Type	Rate Schedule	Alt Codes	CDM Modifiers	CDM Multipliers
Standard		CPT-4 99213		

Txn Modifiers
Override CDM: Yes
Mod 1: 93
Mod 2:
Mod 3:
Mod 4:
Mod 5:

Diagnosis Codes
Dx 1: M47.812
Dx 2: Z98.1
Dx 3: M50.30
Dx 4: M79.2

Rx Data
Prescription Number:
National Drug Code:
Unit of Measure:
Quantity:

Providers
Performing Provider: SARG833
NPI Number: 1225662216
License Number: 45497
Ordering/Referring Provider:
NPI Number:
License Number:

Txn Insurance Order

Ins	Prorated
1 AETNA4	194.00
2 SP	0.00